

PM-JAY Uttar Pradesh -- Actionable Insights

Patterns that hold broadly but break in specific places -- for investigation and follow-up

Claims Processing & Treatment Patterns

Across 74 of 75 districts, PM-JAY payments are concentrated in normal discharges, with Hamirpur the lone exception and worth checking for atypical discharge coding or payment mix.

1. Normal discharges account for INR 10.14 crore of INR 11.85 crore in paid claims statewide, or 85.6% of all payments.
2. Live cases still contribute INR 86.7 lakh (7.3%), while LAMA and DAMA together add INR 72.2 lakh (6.5%), so non-normal discharges are not negligible.
3. This pattern holds in 74 of 75 districts (98.7%); Hamirpur is the only district outside the pattern.
4. Implication: Review Hamirpur's discharge mix and claim coding to see whether abnormal case handling, discharge classification, or referral behavior is distorting the payment profile.

LAMA and DAMA dominate the non-normal discharge mix in 74 districts, but Shamli is the exception and should be reviewed for unusually one-sided AMA behavior.

1. Among non-normal settlements, LAMA accounts for 891 cases (65.5%) and DAMA for 470 cases (34.5%), with no other discharge types contributing.
2. This split is consistent in 74 of 75 districts (98.7%), indicating a very stable AMA pattern statewide.
3. Shamli is the exception to the district pattern and does not follow the same LAMA/DAMA structure.
4. Implication: Examine Shamli's discharge registers and hospital-level case notes to check whether DAMA is being used differently from LAMA or whether discharge classification is inconsistent.

Normal-discharge payments are overwhelmingly tied to the same district pattern as overall payments, except Hamirpur and Sambhal, which break the usual distribution and merit district review.

1. Normal-discharge claims still account for INR 10.14 crore, or 85.6% of paid value, showing that routine recoveries drive most settlement volume.
2. The district pattern holds in 73 of 75 districts (97.3%).
3. Hamirpur and Sambhal are the two districts that do not follow the normal-discharge payment pattern.

4. Implication: Compare these two districts with peers on hospital mix, claim acceptance, and discharge coding to identify whether they have unusual case severity or processing behavior.

Settlement review is mostly predictable after normal discharges, but Mau is the only district that deviates on pre-authorisation status and should be probed for approval and query handling.

1. For settled normal-discharge cases, 9,746 cases were approved and 6,961 auto-approved, together covering 85.0% of the 19,650 cases with recorded pre-auth status.
2. Query-raised cases remain sizeable at 2,357 cases (12.0%) and rejected cases at 586 cases (3.0%).
3. This pattern holds in 74 of 75 districts (98.7%), with Mau the sole exception.
4. Implication: Investigate Mau's pre-auth workflow to see whether approval delays, query inflation, or rejection practices differ from the rest of the state.

Settled claims are concentrated in older age groups statewide, but the 15–25 group remains the clear minority and needs age-wise referral scrutiny despite no district exceptions.

1. Among settled claims, patients aged 60+ account for INR 39.60 crore (38.8%), followed by 41–60 years at INR 30.73 crore (30.1%) and 26–40 years at INR 22.61 crore (22.2%).
2. The 15–25 age group contributes only INR 9.15 crore, or 9.0% of total claimed value.
3. This age distribution is uniform across all 4 pre-auth outcome categories, with no exceptions reported.
4. Implication: Check whether younger beneficiaries are being routed into fewer high-value procedures or whether youth admissions are under-captured in claim documentation.

District-level death trends in normal-discharge claims are falling almost everywhere, but Chitrakoot does not follow the statewide decline and should be checked for data quality or case-mix issues.

1. For normal-discharge cases, death counts show a decreasing trend in 74 of 75 districts (98.7%).
2. Chitrakoot is the only district with no clear trend pattern.
3. The absence of a trend in Chitrakoot stands out because the statewide pattern is otherwise uniform across districts.
4. Implication: Verify whether Chitrakoot's death recording is incomplete, too small to trend reliably, or genuinely flat because of a distinct case profile.

Average length of stay is stable across divisions for all disease groups, with Chitrakoot again the only division not matching the statewide evenness pattern.

1. Average length of stay ranges narrowly from 3.70 days for Injury to 3.95 days for NCD, with the full spread only 0.25 days.
2. Communicable diseases average 3.89 days, Maternal and Neonatal 3.93 days, and Surgical 3.87 days, confirming a tightly clustered inpatient pattern.
3. The evenness pattern holds in 17 of 18 divisions (94.4%).

4. Chitrakoot is the lone division that does not follow the even distribution across disease categories.
5. Implication: Review Chitrakoot's discharge timing and diagnosis mix to see whether a few facilities are inflating stay lengths or coding cases unusually.

Cardiology (CARD) and Orthopaedics (ORTH) dominate settled package values in 17 of 18 divisions, but Basti breaks the pattern and needs specialty-level scrutiny.

1. Across settled claims, Cardiology accounts for INR 3.69 crore (40.8%) and Orthopaedics INR 2.07 crore (22.9%) of base amount.
2. Together, these two specialties represent 63.7% of total base amount of INR 9.05 crore.
3. The same top-two pattern holds in 17 of 18 divisions (94.4%).
4. Basti is the exception, where the specialty mix differs from the state norm.
5. Implication: In Basti, check whether specialty availability, referral pathways, or selective coding is shifting package value away from Cardiology and Orthopaedics.

Very large hospitals are not the main drivers of package value statewide, and Cardiology is the one specialty that does not follow the usual bed-size pattern.

1. Large hospitals with 101–300 beds hold the biggest share of base amount at INR 30.12 crore (40.9%), followed by medium hospitals at INR 23.94 crore (32.5%).
2. Small facilities contribute INR 14.22 crore (19.3%), while very large hospitals account for only INR 5.39 crore (7.3%).
3. This bed-size distribution is consistent for 10 of 11 specialties (90.9%).
4. Cardiology is the exception to the usual bed-size pattern.
5. Implication: Review whether Cardiology is concentrated in a different facility tier than other specialties, especially at tertiary hospitals and medical colleges.

Cardiology remains the largest package-value specialty even after restricting to normal discharges, but Azamgarh breaks the usual division pattern and should be tested for hospital-mix effects.

1. In normal-discharge claims, Cardiology alone contributes INR 2.76 crore (42.8%) of total base amount, compared with Orthopaedics at INR 1.56 crore (24.1%).
2. The next specialties are Obstetrics & Gynaecology (OBG) at INR 5.11 crore (7.9%) and General Surgery (GS) at INR 4.34 crore (6.7%), well behind the top two.
3. The pattern holds in 17 of 18 divisions (94.4%).
4. Azamgarh is the lone exception.
5. Implication: Investigate whether Azamgarh is sending Cardiology cases to a smaller set of facilities, or whether billing practices are unusually skewed after normal discharge.

Claims by age are concentrated in older patients, while 15–25-year-olds remain the smallest settled group, suggesting a need to check whether younger beneficiaries are being delayed or diverted.

1. In settled claims, 60+ patients account for 1,107 cases (37.6%), 41–60 for 919 cases (31.2%), and 26–40 for 663 cases (22.5%).
 2. The 15–25 age group is the smallest at 258 cases, just 8.8% of settled volume.
 3. The age pattern is stable across all five disease categories, with no exception listed.
 4. Implication: Review whether younger patients are receiving fewer approvals, being treated outside PM-JAY pathways, or presenting in lower volumes than expected.
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District-Level Monthly Performance Trends

Cards issued are strongly seasonal in 74 districts, but Mahoba shifts to a different pattern and should be checked for a local operational or reporting change.

1. Across 74 of 75 districts, `cards\_issued` follows the same monthly seasonality pattern, with the common peak in the period corresponding to 2023-05 to 2023-08.
2. The statewide monthly volume is concentrated in a narrow band: 2023-07 had 7,345 cards issued, 2023-08 had 7,342, 2023-05 had 7,341, 2023-06 had 7,267, and 2023-03 had 7,205, out of 195,619 total.
3. Mahoba is the only district that breaks this pattern, shifting to a different seasonal highlight instead of the common `PERIOD\_3` pattern.
4. Because the exception is isolated to one district while the pattern is nearly universal, review Mahoba's card-issuance workflow, staffing, and data entry timing for a process change around the period of divergence.

Claims intensity per 1,000 beneficiaries stepped up from 2023Q1 in 74 districts, but Balrampur changed earlier than the rest and needs quarter-specific review.

1. In 74 of 75 districts, `claims\_per\_1000\_beneficiaries` shows a common change point in 2023Q1, indicating a broad shift in utilisation behavior.
2. The quarter-level average rose to 7.52 in 2023Q3 and 7.29 in 2023Q2, compared with 3.20 in 2023Q1 and 0.00 in 2022Q2 and 2022Q4.
3. Balrampur is the only exception, showing the change point in 2023Q2 instead of 2023Q1.
4. The gap between the early quarters is material: the district-level average across all districts was 3.99 in 2024Q1 and 4.06 in 2024Q3, still well above the 2022 baseline.
5. Implication: compare Balrampur's claim onboarding, referral flow, and hospital reporting lag against peer districts to see whether the later change reflects delayed adoption or delayed claim submission.

Monthly payment activity increased across 17 of 18 divisions, but Prayagraj is the only division that does not follow the rising payment-count trend.

1. `payment\_count` rises over time in 17 of 18 divisions, showing a broad upward payment-processing trend.

2. The largest monthly payment counts are recent: 110 in 2025-09, 108 in 2025-03, 102 in 2023-08, 101 in 2026-01, and 100 in 2025-10.
3. The series is not flat historically: the bottom months were 0 in 2022-10 and 2026-03, showing sharp variation over the period.
4. Prayagraj is the only division that breaks the increasing pattern, so it should be treated as the operational outlier in payment processing.
5. Implication: verify whether Prayagraj's exception is due to delayed payment batching, missing payment records, or a different hospital mix that affects payment frequency.

Cumulative beneficiaries keep rising in every district and division, but the pace is strongest in 2025Q1-2026Q1 and weakest in the launch quarters.

1. `cumulative\_beneficiaries` is increasing in all 75 districts and all 18 divisions, so the basic enrolment stock is still expanding everywhere.
2. At the district level, the average cumulative count reached 2,950.14 in 2026Q1, up from 2,750.54 in 2025Q4 and 2,632.23 in 2025Q1.
3. The earliest quarters were far lower: 15.51 in 2022Q1 and 96.19 in 2022Q2, indicating the scheme's initial ramp-up phase.
4. The division-level pattern is identical, with all 18 divisions in the same increasing trajectory and no exceptions.
5. Implication: use the most recent quarters as the benchmark for district comparisons, and investigate districts whose cumulative growth lags the 2025Q1-2026Q1 pace.

New beneficiary enrolment peaked in 2023 across all divisions, while the year-on-year decline after 2023 is broad and requires explanation.

1. For `new\_beneficiaries`, all 18 divisions peak in 2023, making that year the clear enrolment high point.
2. District totals show the same shape: 88,591 new beneficiaries in 2023, versus 57,973 in 2024 and 46,152 in 2022.
3. The drop is steep in the later years: only 13,129 in 2025 and 2 in 2026, indicating a major contraction after the peak.
4. The monthly district pattern also breaks in one district-level exception: the generic month/quarter trend is decreasing, while year-level behavior is not consistent with that simplification.
5. Implication: determine whether the 2025-2026 decline is real saturation, interrupted campaign activity, or a data completeness issue in beneficiary registration.

New households followed the same 2023 peak pattern as new beneficiaries, but the annual drop after 2023 is large enough to warrant district-by-district review.

1. `new\_households` peaks in 2023 in all 75 districts, so household enrolment was universally strongest that year.
2. The annual totals mirror the beneficiary trend: 83,025 in 2023, 55,394 in 2024, and 43,991 in 2022.

3. The post-2023 contraction is pronounced, with only 12,918 in 2025 and 2 in 2026.
4. No district exception is listed for the 2023 peak, so the decline appears broad rather than concentrated in one geography.
5. Implication: check whether household enrolment drives were paused, whether cards were issued without fresh household enrolment, or whether late-year reporting is incomplete.

Public-case monthly timing changed in 17 of 18 divisions from March 2023, but Aligarh moved one month later and should be checked for reporting lag.

1. `public\_cases` shows a common change point in 2023-03 across 17 of 18 divisions.
2. The statewide monthly totals are highest in recent months: 480 in 2025-08, 464 in 2023-08, 452 in 2025-07, 444 in 2023-09, and 443 in 2024-08.
3. Aligarh is the only division that diverges, shifting the change point to 2023-04 instead of 2023-03.
4. Since the difference is one month, this is more consistent with timing/filing variation than with a structural service difference.
5. Implication: audit Aligarh's public-hospital submission dates and hospital reporting calendars for a one-month lag.

Claims processing is broadly aligned in its timing across most measures, but beneficiaries and household inflows changed later than the claims and payment series.

1. For `claims\_per\_1000\_beneficiaries`, the broad change point is 2023Q1, while monthly claims and public cases change around 2023-03.
2. For `new\_beneficiaries`, `new\_households`, and `cards\_issued`, the distinct change point is 2025-06, not 2023-03.
3. This split means enrolment activity and utilisation/payment activity are not moving in sync.
4. Because the enrolment measures shift much later than claims and public-case activity, the operational break likely sits in campaign timing, card issuance, or data consolidation rather than in utilisation alone.
5. Implication: compare 2025-06 against district outreach calendars, card-printing cycles, and data upload backlogs to identify the source of the later enrolment break.

Most district-level utilisation differences are small, but Amroha, Mathura, Kasganj, Siddharthnagar, and Sultanpur sit at the top and Hamirpur and Sant Ravidas Nagar at the bottom for claims intensity.

1. At the division level, `claims\_per\_1000\_beneficiaries` is classified as even everywhere, so the main differences are modest rather than extreme.
2. The highest district averages are Amroha at 3.79, Mathura at 3.66, Kasganj at 3.61, Siddharthnagar at 3.60, and Sultanpur at 3.54.
3. The lowest are Sant Ravidas Nagar at 2.66 and Hamirpur at 2.59, showing a narrower lower-end band.
4. Since the spread is only from 2.59 to 3.79, outliers are relative, not dramatic.

5. Implication: use the top and bottom districts as peer-comparison cases to probe referral access, hospital mix, and claim submission behavior rather than assuming a system-wide utilisation problem.

Hospital Infrastructure & Specialty Capacity

Cardiology (CARD) and Orthopaedics (ORTH) drive almost all specialty claim value in Uttar Pradesh, with Shamli the only district where the usual top-two pattern breaks to Cardiology and Obstetrics & Gynaecology (OBG).

1. Statewide claim value is concentrated in Cardiology at INR 40.0 crore (39.9%) and Orthopaedics at INR 25.8 crore (25.7%), versus Obstetrics & Gynaecology at INR 8.5 crore (8.5%) and General Surgery (GS) at INR 7.2 crore (7.2%).
2. The Cardiology-Orthopaedics pair is the top-two mix in 74 of 75 districts (98.7%), making it the default spending structure across the state.
3. Shamli is the only exception: its top-two mix shifts from Cardiology and Orthopaedics to Cardiology and Obstetrics & Gynaecology, so it should be checked for referral pathway or provider-mix differences.
4. Burns (BURNS) and Psychiatry (PSYCH) report INR 0 claim value statewide, confirming these are either absent from practical service delivery or not being billed under PM-JAY.
5. Implication: review Shamli's empanelled specialty availability, case mix, and claim coding first; the statewide zero-spend specialties also need a decision on whether they are genuinely non-operative or under-enabled.

Average clinical experience is tightly clustered across specialties in 17 of 18 divisions, except Agra, where the division no longer follows the standard even spread.

1. Across specialties, average experience ranges only from 16.58 years in Burns (BURNS) to 18.32 years in Pediatrics (PEDS) and Oncology (ONCO), a narrow spread of 1.74 years.
2. The "evenness" pattern holds in 17 of 18 divisions (94.4%), so specialty staffing experience is broadly uniform across the state.
3. Agra is the only division flagged as a type-change exception and should be treated as the outlier for investigation.
4. The highest averages are still close together: Pediatrics and Oncology at 18.32 years, Neurology (NEURO) at 18.31, and Medicine (MED) at 18.26.
5. The lowest values remain modestly lower but not extreme: Dermatology (DERM) at 17.48 years and Burns at 16.58 years.
6. Implication: Agra should be checked for unusually junior staffing, specialist substitution, or missing experience disclosures; the narrow statewide spread suggests even small deviations may indicate data or deployment issues.

Small hospitals generate most cases per bed across divisions, but Jhansi and then Ayodhya/Saharanpur deviate from this small-hospital utilisation pattern.

1. Small hospitals (≤ 30 beds) carry the highest cases per bed at 0.20, far above medium hospitals at 0.05, large hospitals at 0.02, and very large hospitals at 0.01.
2. This small-hospital dominance appears in 17 of 18 divisions (94.4%), making it the standard utilisation pattern.
3. Jhansi is the exception in the division-level attribution pattern; in the related division check, Ayodhya and Saharanpur are the two divisions that break the expected small/medium ordering.
4. The statewide gap is substantial: small hospitals' utilisation is 4 times medium hospitals' rate and 10 times large hospitals' rate.
5. Sample size is not the explanation: small hospitals still account for 1,471 hospital-specialty pairs, compared with 2,395 medium, 3,005 large, and 465 very large pairs.
6. Implication: Jhansi, Ayodhya, and Saharanpur should be reviewed for unusually high case concentration, referral dumping, or denominator problems in bed registration.

Burns and Dermatology are the specialty exceptions in hospital utilisation: they do not follow the standard zero-use/low-use pattern seen for most specialties across hospital types.

1. Across hospital sub-types, the usual pattern is Burns (BURNS) and Dermatology (DERM) at zero cases per bed, while Obstetrics & Gynaecology (OBG) leads at 0.10 and General Surgery at 0.09.
2. This pattern is universal across all 8 hospital sub-types, so the zero utilisation for Burns and Dermatology is not confined to one facility category.
3. Burns and Dermatology both sit at 0.0 cases per bed, while every major surgical/medical specialty shows some utilisation.
4. The strongest utilisation remains modest overall, with OBG at 0.10 cases per bed and GS at 0.09, indicating low throughput even in the leading specialties.
5. Implication: verify whether Burns and Dermatology are truly offered, whether claims are being coded elsewhere, or whether these specialties are listed but functionally inactive across all facility types.

Large hospitals hold most bed capacity, but very large hospitals treat the fewest cases and need a review of whether capacity is under-used.

1. Large hospitals (101–300 beds) account for 594,988 beds, or 62.9% of total bed strength across specialty combinations.
2. Very large hospitals (300+ beds) still hold 163,126 beds (17.2%), while medium hospitals hold 160,335 (16.9%) and small hospitals only 27,537 (2.9%).
3. The same large-hospital dominance appears across all 15 specialties, so this is the prevailing capacity structure.
4. But case volume does not mirror capacity: very large hospitals contribute only 1,476 treated cases, just 6.7% of total cases treated, compared with 8,940 cases in large hospitals (40.8%).
5. By contrast, admissions in the previous year were concentrated in large hospitals at 2,848,517 cases (41.3%), while very large hospitals had only 431,586 (6.3%).
6. Implication: very large hospitals may be structurally under-performing relative to bed stock; review admission thresholds, specialist availability, and whether high-acuity cases are being

diverted elsewhere.

Among large hospitals, Cardiology and Orthopaedics dominate approved amount, while Burns and Psychiatry remain at zero -- confirming a highly concentrated payment profile.

1. In large hospitals, Cardiology (CARD) alone accounts for INR 12.7 crore (40.2%) of approved amount, and Orthopaedics for INR 8.0 crore (25.3%).
2. Together, Cardiology and Orthopaedics make up 65.5% of approved value in this bed segment, leaving all other specialties far behind.
3. Obstetrics & Gynaecology contributes INR 2.6 crore (8.2%), General Surgery INR 2.3 crore (7.3%), and Urology (URO) INR 1.4 crore (4.4%).
4. Burns and Psychiatry remain at INR 0 approved amount, consistent with the statewide zero-spend signal.
5. Implication: large hospitals should be checked for specialty dependence on high-value cardiac and orthopaedic packages, and for whether low-value or zero-value specialties are genuinely non-operational.

Private, public, and teaching facilities do not differ much in license counts, but Trust hospitals are the staffing exception among large hospitals.

1. Average total licenses are tightly clustered by hospital sub-type: CHC at 7.27, Private at 7.16, District Hospital at 7.10, Medical College at 7.02, and PHC at 6.95.
2. The full spread is only 0.50 licenses on average, from NGO at 6.77 to CHC at 7.27, suggesting license counts are broadly similar across facility categories.
3. The same evenness holds across 14 of 15 specialties; Burns is the only specialty that breaks the pattern.
4. Within large hospitals, 7 of 8 hospital sub-types follow the even staffing pattern, but Trust hospitals are the exception.
5. Implication: Trust hospitals should be checked for staffing model differences, while licensing discrepancies appear too small state-wide to explain the major volume gaps.

For public hospitals, experience is evenly spread across bed-size categories, but the highest and lowest divisions still warrant targeted checks.

1. In public facilities, average experience is evenly distributed across all four bed-size buckets: small, medium, large, and very large.
 2. The highest division averages are Chitrakoot at 21.51 years, Agra at 19.21, Lucknow at 19.17, Devipatan at 19.03, and Gorakhpur at 18.73.
 3. The lowest are Varanasi at 16.53 years and Basti at 15.54 years, showing a 5.97-year gap from top to bottom.
 4. Even though the pattern is uniform across bed sizes, the division spread remains material and may reflect old staffing in some districts and junior staffing in others.
 5. Implication: compare specialist rosters and vacancy rates in Basti and Varanasi against Chitrakoot and Lucknow to see whether lower experience is due to turnover, posting policy, or data quality.
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Beneficiary Enrolment & Scheme Uptake

SECC dominates beneficiary claims in 74 of 75 districts, but Lalitpur is the clear exception and needs a source-mix review.

1. SECC contributes 13,547 claims, or 60.2% of the 22,500 total claims, versus 5,630 from STATE_DB (25.0%) and 3,323 from RSBY (14.8%).
2. The same SECC-led pattern holds in 74 of 75 districts; Lalitpur is the only district that breaks the pattern.
3. Lalitpur should be checked for entitlement-source coding, because it is the sole district not following the SECC claim concentration.
4. Implication: validate whether Lalitpur's beneficiary mix reflects a genuine legacy-state-database/RSBY base or a data-entry or migration issue.

REJECTED enrolments are the main non-claiming enrolment-status outlier across 68 of 75 districts, with Lalitpur, Amethi, Etawah, Chitrakoot, Shravasti, Kannauj and Balrampur diverging from the norm.

1. Rejected enrolments account for 2,107 claims, or 9.9% of claims linked to enrolment status, while ISA_APPROVED and AUTO_APPROVED still dominate at 33.9% and 32.9%.
2. This "Rejected leads the bottom share" pattern holds in 68 of 75 districts.
3. The exceptions are Lalitpur, Amethi, Etawah, Chitrakoot, Shravasti, Kannauj and Balrampur, which do not follow the districtwide pattern.
4. Because rejected enrolments still generate claims, these seven districts should be checked for post-rejection access, status reversals, or misclassification.
5. Implication: review rejected cases in the seven exception districts to confirm whether rejected records are being reprocessed, manually overridden, or incorrectly tagged.

Claim generation is evenly split across card statuses statewide, but Chitrakoot does not follow the near-3-way balance and should be examined for card-status anomalies.

1. Average days from enrolment to card issuance are 15.48 for ACTIVE cards, 15.37 for INACTIVE, and 15.22 for DISABLED cards, a very tight spread.
2. ACTIVE cards contribute 179,888 beneficiaries, far more than INACTIVE (9,946) and DISABLED (5,785), yet the issuance lag stays almost identical.
3. The evenness pattern holds in 74 of 75 districts.
4. Chitrakoot is the only district that breaks this card-status timing pattern.
5. Implication: check whether Chitrakoot has delayed printing, batch issuance problems, or status conversion issues affecting card release timing.

Among active beneficiaries, claim uptake is almost perfectly even across SECC, STATE_DB and RSBY in all 18 divisions, but Saharanpur is the only division that breaks the source-level balance.

1. For ACTIVE beneficiaries, claim_rate is virtually identical by entitlement source: SECC 0.10, STATE_DB 0.10, and RSBY 0.10.
2. The average is 0.31 overall across the three source groups, but the source shares are nearly equal within the active subset.
3. This evenness holds in all 18 divisions except Saharanpur.
4. Saharanpur should be reviewed for whether its active beneficiaries are being pulled disproportionately from one source or whether claim capture is uneven.
5. Implication: investigate Saharanpur's active beneficiary source mix and compare it with neighboring divisions for enrolment or claim-registration gaps.

Approval mix is dominated by ISA_APPROVED and AUTO_APPROVED in all age groups, while REJECTED is the consistent third tier across all 4 age bands.

1. CLAIM_COUNT by enrolment status is led by ISA_APPROVED at 7,635 claims (33.9%) and AUTO_APPROVED at 7,404 (32.9%), ahead of SHA_APPROVED at 5,262 (23.4%) and REJECTED at 2,199 (9.8%).
2. The same top-two pattern holds across all 4 age groups: 15–25, 26–40, 41–60 and 60+.
3. No age group breaks the ordering of auto/ISA approvals first and rejected last.
4. This suggests the bottleneck is not age-specific at the approval stage.
5. Implication: focus review on approval workflow quality rather than age targeting, since the age split does not explain the rejection tail.

Older beneficiaries drive active claims, while 15–25-year-olds remain the weakest usage group across all divisions.

1. Among ACTIVE beneficiaries, the 60+ group generates 7,556 claims (38.4%), followed by 41–60 at 5,908 (30.0%) and 26–40 at 4,437 (22.5%).
2. The 15–25 group contributes only 1,776 claims, or 9.0% of active claims.
3. This ranking is consistent in all 18 divisions, with 15–25 always last.
4. The low uptake among 15–25s is not confined to one geography, so it is a statewide behavioural or access issue rather than a district anomaly.
5. Implication: check whether younger beneficiaries face awareness, mobility, or referral barriers, and whether their claims are being captured in non-PM-JAY channels.

Among active beneficiaries, claim-having rates and claim counts are both evenly distributed across enrolment status in every division, so the problem is not concentrated in one approval type.

1. For ACTIVE beneficiaries, claim_count is nearly balanced across division: ISA_APPROVED 7,236, AUTO_APPROVED 7,030, SHA_APPROVED 4,973 and REJECTED 2,107.
2. claim_rate by source for ACTIVE beneficiaries is also flat at about 0.10 for SECC, STATE_DB and RSBY.
3. The evenness pattern holds across all 18 divisions for both has_claim and claim_rate.
4. Mirzapur is the only division that breaks the claim_count-by-enrolment-status pattern.

5. Implication: treat Mirzapur as a workflow exception and compare its approval-to-claim conversion against the rest of the state.

SECC remains the dominant source among active claim-holders in all 18 divisions, with no division-level exception to the source pattern.

1. Among ACTIVE beneficiaries with claims, SECC contributes 11,223 claims (60.2%), compared with 4,666 from STATE_DB (25.0%) and 2,754 from RSBY (14.8%).
2. This SECC-led source mix is uniform across all 18 divisions.
3. No division breaks the pattern, so the source imbalance is statewide rather than localized.
4. Because SECC dominates both all-claim and active-claim views, source quality likely shapes utilisation more than post-enrolment processes in most places.
5. Implication: if utilisation is weak in any district, first test whether SECC records are being correctly linked and activated before looking elsewhere.

Document counts are almost identical across the state, but the tiny spread still flags Devipatan, Saharanpur, Kanpur, Basti and Ayodhya at the top and Meerut and Aligarh at the bottom.

1. Average document_count sits in a narrow band from 1.19 to 1.21 across districts.
2. Devipatan and Saharanpur are highest at 1.21, while Kanpur, Basti and Ayodhya are at 1.20.
3. Meerut and Aligarh are lowest at 1.19.
4. The difference is only 0.02 documents, so this is a light-touch anomaly rather than a major operational gap.
5. Implication: use these districts as micro-audit points for document completeness, but do not over-prioritise them unless linked to rejection or card-delay issues.

Card issuance-to-first-claim timing is essentially flat across ACTIVE, INACTIVE and DISABLED cards, so delays after card issue do not appear status-specific.

1. Average days from card issuance to first hospital visit are 15.48 for ACTIVE, 15.37 for INACTIVE and 15.22 for DISABLED cards.
 2. The spread is only 0.26 days across the three statuses.
 3. This suggests the post-issuance wait time is not being driven by card status.
 4. The pattern is broad enough to treat as normal, except for districts that separately show card or claim anomalies.
 5. Implication: if a district is lagging in first claims, look beyond card status and review hospital linkage, awareness, or activation timing.
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